

Billing and Coding: Hypoglossal Nerve Stimulation for Obstructive Sleep Apnea

A58075

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Palmetto GBA	A and B MAC	10111 - MAC A	J - J	Alabama
Palmetto GBA	A and B MAC	10112 - MAC B	J - J	Alabama
Palmetto GBA	A and B MAC	10211 - MAC A	J - J	Georgia
Palmetto GBA	A and B MAC	10212 - MAC B	J - J	Georgia
Palmetto GBA	A and B MAC	10311 - MAC A	J - J	Tennessee
Palmetto GBA	A and B MAC	10312 - MAC B	J - J	Tennessee
Palmetto GBA	A and B and HHH MAC	11201 - MAC A	J - M	South Carolina

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Palmetto GBA	A and B and HHH MAC	11202 - MAC B	J - M	South Carolina
Palmetto GBA	A and B and HHH MAC	11301 - MAC A	J - M	Virginia
Palmetto GBA	A and B and HHH MAC	11302 - MAC B	J - M	Virginia
Palmetto GBA	A and B and HHH MAC	11401 - MAC A	J - M	West Virginia
Palmetto GBA	A and B and HHH MAC	11402 - MAC B	J - M	West Virginia
Palmetto GBA	A and B and HHH MAC	11501 - MAC A	J - M	North Carolina
Palmetto GBA	A and B and HHH MAC	11502 - MAC B	J - M	North Carolina

Article Information

General Information

Article ID

A58075

Article Title

Billing and Coding: Hypoglossal Nerve Stimulation for Obstructive Sleep Apnea

Article Type

Billing and Coding

Original Effective Date

06/21/2020

Revision Effective Date

05/21/2026

Revision Ending Date

N/A

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N/A

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CMS National Coverage Policy

Title XVIII of the Social Security Act, §1833(e) prohibits Medicare payment for any claim which lacks the necessary information to process the claim.

CMS Internet-Only Manual, Pub. 100-04, Medicare Claims Processing Manual, Chapter 20 Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS)

Article Guidance

Article Text

The information in this article contains billing, coding or other guidelines that complement the Local Coverage Determination (LCD) for Hypoglossal Nerve Stimulation for Obstructive

Sleep Apnea L38276.

General Guidelines for Claims submitted for Part A or Part B Services:

Procedure codes may be subject to National Correct Coding Initiative (NCCI) edits or OPPS packaging edits. Refer to NCCI and OPPS requirements prior to billing Medicare.

For services requiring a referring/ordering physician, the name and NPI of the referring/ordering physician must be reported on the claim.

A claim submitted without a valid ICD-10-CM diagnosis code will be returned to the provider as an incomplete claim under Section 1833(e) of the Social Security Act.

The diagnosis code(s) must best describe the patient's condition for which the service was performed. For diagnostic tests, report the result of the test if known; otherwise, the symptoms prompting the performance of the test should be reported.

Advance Beneficiary Notice of Non-coverage (ABN) Modifier Guidelines

An ABN may be used for services which are likely to be non-covered, whether for medical necessity or for other reasons. Refer to CMS Publication 100-04, Medicare Claims Processing Manual, Chapter 30, for complete instructions.

Effective from April 1, 2010, non-covered services should be billed with modifier –GA, –GX, –GY, or –GZ, as appropriate.

The –GA modifier (“Waiver of Liability Statement Issued as Required by Payer Policy”) should be used when physicians, practitioners, or suppliers want to indicate that they anticipate that Medicare will deny a specific service as not reasonable and necessary and they **do have** an ABN signed by the beneficiary on file. Modifier GA applies only when services will be denied under reasonable and necessary provisions, sections 1862(a)(1), 1862(a)(9), 1879(e), or 1879(g) of the Social Security Act. Effective April 1, 2010, Part A MAC systems will automatically deny services billed with modifier GA. An ABN, Form CMS-R-131, should be signed by the beneficiary to indicate that he/she accepts responsibility for payment. The –GA modifier may also be used on assigned claims when a patient refuses to sign the ABN and the latter is properly witnessed. For claims submitted to the Part A MAC, occurrence code 32 and the date of the ABN is required.

Modifier GX (“Notice of Liability Issued, Voluntary Under Payer Policy”) should be used when the beneficiary has signed an ABN, and a denial is anticipated based on provisions *other* than medical necessity, such as statutory exclusions of coverage or technical issues. An ABN is not required for these denials, but if non-covered services are reported with modifier GX, Part A MAC systems will automatically deny the services.

The –GZ modifier should be used when physicians, practitioners, or suppliers want to indicate that they expect that Medicare will deny an item or service as not reasonable and necessary and they **have not** had an ABN signed by the beneficiary.

If the service is statutorily non-covered or without a benefit category, submit the appropriate CPT/HCPCS code with the -GY modifier. An ABN is not required for these denials, and the limitation of liability does not apply for beneficiaries. Services with modifier GY will automatically deny.

New implantation of a Hypoglossal Nerve Stimulator (HNS) for treatment of OSA code one of the following:

- **CPT® code 64582** - Open implantation of hypoglossal nerve neurostimulator array, pulse generator, and distal respiratory sensor electrode or electrode array
- **HCPCS code C8007** - Open implantation of hypoglossal nerve neurostimulator array and pulse generator, not requiring an insertion of a separate distal respiratory sensor electrode or electrode array

Revision or replacement during the life cycle of HNS components for treatment of OSA is reported with one of the following (as applicable):

- **CPT® code 64583** - Revision or replacement of hypoglossal nerve neurostimulator array and distal respiratory sensor electrode or electrode array, including connection to existing pulse generator
- **HCPCS code C8008** - Revision or replacement of hypoglossal nerve neurostimulator array including connection to existing pulse generator

*Append modifier 52 in instances where only a portion of the device listed in the description is revised (e.g., a lead revision procedure addressing only one lead).

Removal of component(s) of an HNS for treatment of OSA is reported with one of the following (as applicable):

- **CPT® code 64584** - Removal of hypoglossal nerve neurostimulator array, pulse generator, and distal respiratory sensor electrode or electrode array
- **HCPCS code C8009** - Removal of hypoglossal nerve neurostimulator array and pulse generator

*Append modifier 52 in instances where only a portion of the device listed in the description is removed (e.g., removal of generator only or both generator and stimulation lead or breathing sensor lead only or stimulation lead only).

Documentation Requirements

The patient's medical record must contain documentation that fully supports the medical necessity for services included within the *Coverage Indications, Limitations and/or Medical*

Necessity section within the related LCD. This documentation includes, but is not limited to, appropriate use criteria, relevant medical history, physical examination, and results of pertinent diagnostic tests or procedures.

The patient’s medical record must document all of the following:

- Symptoms
- Polysomnography reporting
- Drug Induced Sleep Endoscopy (DISE) reporting-CPT® code 42975
- Body mass index (BMI)

Replacement or revision of an implanted neurostimulator pulse generator system (with or without lead changes) for hypoglossal nerve stimulation is considered medically necessary in an individual when the implantation occurred because the LCD coverage criteria were met.

Utilization Guidelines

In accordance with CMS Ruling 95-1 (V), utilization of these services should be consistent with locally acceptable standards of practice.

Coding Information

CPT/HCPCS Codes

Group 1 (6 Codes)

Group 1 Paragraph

Note: CPT® codes related to component(s) removal, replacement or revision do not require a dual diagnosis.

Group 1 Codes

Code	Description
64582	Opn mpltj hpglsl nstm ary pg
64583	Rev/rplct hpglsl nstm ary pg

Code	Description
64584	Rmvl hpglsl nstim ary pg
C8007	Opn mplnt hpgls ns ary ps gn
C8008	Rv/rpl hpgls ns inc cnt pg
C8009	Rmv hpgls ns ary and pg

CPT/HCPCS Modifiers

Group 1 (5 Codes)

Group 1 Paragraph

N/A

Group 1 Codes

Code	Description
52	REDUCED SERVICES: UNDER CERTAIN CIRCUMSTANCES A SERVICE OR PROCEDURE IS PARTIALLY REDUCED OR ELIMINATED AT THE PHYSICIAN'S DISCRETION. UNDER THESE CIRCUMSTANCES THE SERVICE PROVIDED CAN BE IDENTIFIED BY ITS USUAL PROCEDURE NUMBER AND THE ADDITION OF THE MODIFIER -52, SIGNIFYING THAT THE SERVICE IS REDUCED. THIS PROVIDES A MEANS OF REPORTING REDUCED SERVICES WITHOUT DISTURBING THE IDENTIFICATION OF THE BASIC SERVICE. MODIFIER CODE 09952 MAY BE USED AS AN ALTERNATIVE TO MODIFIER -52. NOTE: FOR HOSPITAL OUTPATIENT REPORTING OF A PREVIOUSLY SCHEDULED PROCEDURE/SERVICE THAT IS PARTIALLY REDUCED OR CANCELLED AS A RESULT OF EXTENUATING CIRCUMSTANCES OR THOSE THAT THREATEN THE WELL-BEING OF THE PATIENT PRIOR TO OR AFTER ADMINISTRATION OF ANESTHESIA, SEE MODIFIERS -73 AND -74 (SEE MODIFIERS APPROVED FOR ASC HOSPITAL OUTPATIENT USE).
GA	WAIVER OF LIABILITY STATEMENT ISSUED AS REQUIRED BY PAYER POLICY, INDIVIDUAL CASE
GX	NOTICE OF LIABILITY ISSUED, VOLUNTARY UNDER PAYER POLICY
GY	ITEM OR SERVICE STATUTORILY EXCLUDED, DOES NOT MEET THE DEFINITION OF ANY MEDICARE BENEFIT OR, FOR NON-MEDICARE INSURERS, IS NOT A CONTRACT BENEFIT
GZ	ITEM OR SERVICE EXPECTED TO BE DENIED AS NOT REASONABLE AND NECESSARY

ICD-10-CM Codes that Support Medical Necessity

Group 1 (1 Code)

Group 1 Paragraph

Use of these codes does not guarantee reimbursement. The patient's medical record must document that the coverage criteria in the related LCD have been met.

Dual diagnosis requirement

Coverage for new insertion of a hypoglossal nerve stimulation device on patients who meet coverage criteria set forth in LCD L38276 must include both a primary ICD-10-CM diagnosis code indicating the reason for the procedure, and a secondary ICD-10-CM diagnosis code indicating the BMI is less than 35 kg/m² as set forth in the *Coverage Indications, Limitations and/or Medical Necessity* section of the related LCD.

Report a primary diagnosis code from *ICD-10-CM Codes that Support Medical Necessity Group 1: Codes* and a secondary diagnosis code from *ICD-10-CM Codes that Support Medical Necessity Group 2: Codes*.

Note: CPT® code 64584 and C8009 will have no diagnosis to procedure code restriction at this time.

Primary Diagnosis**Group 1 Codes**

Code	Description
G47.33	Obstructive sleep apnea (adult) (pediatric)

Group 2 (16 Codes)**Group 2 Paragraph****Secondary Diagnosis****Group 2 Codes**

Code	Description
Z68.1	Body mass index [BMI] 19.9 or less, adult
Z68.20	Body mass index [BMI] 20.0-20.9, adult
Z68.21	Body mass index [BMI] 21.0-21.9, adult
Z68.22	Body mass index [BMI] 22.0-22.9, adult

Code	Description
Z68.23	Body mass index [BMI] 23.0-23.9, adult
Z68.24	Body mass index [BMI] 24.0-24.9, adult
Z68.25	Body mass index [BMI] 25.0-25.9, adult
Z68.26	Body mass index [BMI] 26.0-26.9, adult
Z68.27	Body mass index [BMI] 27.0-27.9, adult
Z68.28	Body mass index [BMI] 28.0-28.9, adult
Z68.29	Body mass index [BMI] 29.0-29.9, adult
Z68.30	Body mass index [BMI] 30.0-30.9, adult
Z68.31	Body mass index [BMI] 31.0-31.9, adult
Z68.32	Body mass index [BMI] 32.0-32.9, adult
Z68.33	Body mass index [BMI] 33.0-33.9, adult
Z68.34	Body mass index [BMI] 34.0-34.9, adult

ICD-10-CM Codes that DO NOT Support Medical Necessity

Group 1

Group 1 Paragraph

All those not listed under the *ICD-10-CM Codes that Support Medical Necessity* section of this article.

Group 1 Codes

N/A

ICD-10-PCS Codes

N/A

Additional ICD-10 Information

N/A

Bill Type Codes

Contractors may specify Bill Types to help providers identify those Bill Types typically used to report this service. Absence of a Bill Type does not guarantee that the article does not apply to that Bill Type. Complete absence of all Bill Types indicates that coverage is not influenced by Bill Type and the article should be assumed to apply equally to all claims.

N/A

Revenue Codes

Contractors may specify Revenue Codes to help providers identify those Revenue Codes typically used to report this service. In most instances Revenue Codes are purely advisory. Unless specified in the article, services reported under other Revenue Codes are equally subject to this coverage determination. Complete absence of all Revenue Codes indicates that coverage is not influenced by Revenue Code and the article should be assumed to apply equally to all Revenue Codes.

N/A

Other Coding Information

N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation
05/21/2026	R9	Under ICD-10-CM Codes that Support Medical Necessity Group 1: Paragraph added "Note: CPT® code 64584 and C8009 will have no diagnosis to procedure code restriction at this time."
01/01/2026	R8	Under Article Text revised text to reflect the addition of HCPCS codes C8007, C8008 and C8009. Under CPT/HCPCS Codes Group 1: Codes added C8007, C8008 and C8009. This revision is due to the 2026 Q2 CPT/HCPCS Code Update and is retroactive effective for dates of service on or after 1/1/26.
01/01/2026	R7	Under Article Text revised text to reflect the deletion of CPT® codes 61886, 61888, 64568, 64569 and 64570. Under CPT/HCPCS Codes Group 1: Codes deleted 61886, 61888, 64568, 64569 and 64570.
01/01/2025	R6	Under ICD-10-CM Codes that Support Medical Necessity Group 1: Paragraph removed the sentence "Medicare is establishing the following limited coverage for CPT® code 64582 ." This revision is retroactive effective for dates of service on or after 1/1/25.
01/01/2025	R5	Under Article Text revised text to reflect the addition of CPT® codes 64568, 64569 and 64570. Under CPT/HCPCS Codes Group 1: Codes added 64568, 64569 and 64570. This revision is retroactive effective for dates of service on or after 1/1/25.
01/01/2022	R4	Under Article Text revised bulleted descriptions of CPT® codes and the explanation for the use of modifiers.

Revision History Date	Revision History Number	Revision History Explanation
		Formatting was adjusted and acronyms were defined throughout the article where applicable. Under <i>CPT/HCPCS Codes Group 1: Codes</i> added codes 64582, 64583 and 64584, and deleted codes 64568, 64569, 64570, 0466T, 0467T, and 0468T. Under <i>CPT/HCPCS Modifiers Group 1: Codes</i> added modifier 52. Under <i>ICD-10-CM Codes that Support Medical Necessity Group 1: Paragraph</i> revised verbiage to support annual CPT/HCPCS code revisions. This revision is due to the Annual CPT/HCPCS update and is effective on 1/1/22.
10/01/2021	R3	Under <i>ICD-10 Codes that Support Medical Necessity Group 2: Codes</i> the description was revised for code Z68.30. This revision is due to the Annual ICD-10 update and is effective on 10/1/21.
10/01/2020	R2	Under <i>Covered ICD-10 Codes Group 2: Codes</i> the description was revised for codes Z68.1, Z68.20, Z68.21, Z68.22, Z68.23, Z68.24, Z68.25, Z68.26, Z68.27, Z68.28, Z68.29, Z68.30, Z68.31, Z68.32, Z68.33, and Z68.34. This revision is due to the Annual ICD-10 Code Update and is effective on 10/1/20.
06/21/2020	R1	Under <i>Article Text General Guidelines for Claims submitted for Part A or Part B Services</i> added verbiage to eleventh paragraph to read “New implantation of a Hypoglossal Nerve Stimulator (HNS) for treatment of OSA utilizes 2 CPT® codes:”. Under <i>Article Text Revision or replacement</i> added verbiage to subheading to read “Revision or replacement during the life cycle of HNS components for treatment of OSA is reported with one or more of the following (as applicable)”. Under <i>Article Text Revision or replacement</i> added CPT® codes 64569, 61886, and 61888 with descriptions. Under <i>Article Text Removal</i> added verbiage to subheading to read “Removal of component(s) of a HNS for treatment of OSA is reported with one or more of the following (as applicable)”. Under <i>Article Text Removal</i> added CPT®

Revision History Date	Revision History Number	Revision History Explanation
		codes 64570 and 61888 with descriptions. Under Article Text Documentation Requirements added verbiage to read “Replacement or revision of an implanted neurostimulator pulse generator system (with or without lead changes) for hypoglossal nerve stimulation is considered medically necessary in an individual when the implantation occurred because the LCD coverage criteria were met”. Under CPT/HCPCS Codes Group 1: Paragraph added verbiage to subheading Note to read “CPT® codes related to component(s) removal, replacement or revision do not require a dual diagnosis”. Under CPT/HCPCS Codes Group 1: Codes added 61886 and 61888. Under ICD-10 Codes that Support Medical Necessity Group 1: Paragraph Dual diagnosis requirement added verbiage to read “Coverage for new insertion of a hypoglossal nerve stimulation device on patients who meet coverage criteria set forth in LCD L38276 must include both a primary ICD-10-CM diagnosis code indicating the reason for the procedure, and a secondary ICD-10-CM diagnosis code indicating the Body Mass Index (BMI) is less than 35 kg/m2 as set forth in the Coverage Indications, Limitations and/or Medical Necessity section of the related LCD” removed verbiage “and for 0467T” from third paragraph. Formatting and typographical errors were corrected throughout the LCD.

Associated Documents

Related Local Coverage Documents

LCDs

[L38276 - Hypoglossal Nerve Stimulation for Obstructive Sleep Apnea](#) 

Related National Coverage Documents

NCDs

N/A

Statutory Requirements URLs

N/A

Rules and Regulations URLs

N/A

CMS Manual Explanations URLs

N/A

Other URLs

N/A

Public Versions

Updated On	Effective Dates	Status	
05/15/2026	05/21/2026 - N/A	Currently in Effect	You are here
04/10/2026	01/01/2026 - 05/20/2026	Superseded	View
12/22/2025	01/01/2026 - N/A	Superseded	View
09/18/2025	01/01/2025 - 12/31/2025	Superseded	View
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Keywords

- Hypoglossal
- Nerve Stimulation
- Obstructive Sleep Apnea

